INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab + ipilimumab · first-line colorectal

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00900
Version 1a

> Cycles 1–4: nivolumab 240 mg, then ipilimumab 1 mg/kg, Day 1 every 21 days.

Treatment stages

01 **Cycles 1–4: two immunotherapies**
Every 21 days

02 **Cycle 5 onwards: nivolumab alone**
Team-selected maintenance schedule

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for new breathlessness, persistent diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, spreading or blistering rash, severe headache, marked weakness, confusion, unusual thirst or passing much more urine. Severe breathing difficulty, chest pain, collapse or seizure needs emergency care. Effects can begin after treatment ends.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

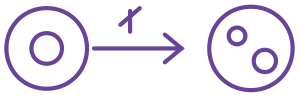
Three medicines. Different jobs.

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR deoxyribonucleic acid (the cell's genetic material) (DNA)-repair feature. This is a first-line treatment for unresectable or metastatic cancer.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks PD-1

Nivolumab blocks PD-1, a brake on immune T cells. This can help immune cells attack cancer, but can also inflame healthy organs.

Immune T cell → PD-1 brake blocked → Immune attack

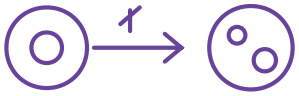
What to watch for

Report new breathing, bowel, hormone, skin, heart or nerve symptoms promptly.

IMMUNOTHERAPY

Ipilimumab

Yervoy



Blocks CTLA-4

Ipilimumab blocks another immune brake, CTLA-4, helping activate T cells. Combining two immune medicines can increase anti-cancer activity and also the risk of serious immune inflammation.

Immune activation → CTLA-4 brake blocked → T-cell response

What to watch for

Report diarrhoea, abdominal pain, skin changes, headache, weakness or other new symptoms immediately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your cancer and treatment aim

A validated tumour result helps select this treatment.

MSI-H and dMMR

These describe a feature of the cancer's DNA-repair system. The result helps identify colorectal cancers that may respond to immunotherapy. Ask whether a genetics assessment is relevant.

Choices and goals

Discuss the likely benefit, uncertainty, other treatment options, clinical trials and symptom-focused care in relation to your priorities.

How the two medicines work

They act at two different immune-control points.

Why side effects need prompt assessment

Immune activity may inflame healthy organs. The combination can cause serious effects, and symptoms may continue or begin after ipilimumab or all treatment stops. Pauses, steroids or permanent discontinuation may be needed.

Evidence for this colorectal setting

CheckMate 8HW compared nivolumab plus ipilimumab with chemotherapy in first-line MSI-H/dMMR metastatic colorectal cancer.

The randomised trial showed longer progression-free survival with the combination than chemotherapy; results for groups cannot predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment → review → next steps

Treatment stages

01 **Cycles 1–4: two immunotherapies**
Every 21 days

02 **Cycle 5 onwards: nivolumab alone**
Team-selected maintenance schedule

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 21 DAYS

Cycles 1–4: two immunotherapies

C1 C2 C3 C4

Nivolumab 240 mg followed by ipilimumab 1 mg/kg on Day 1. Both are given as infusions. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review.

TEAM-SELECTED MAINTENANCE SCHEDULE

Cycle 5 onwards: nivolumab alone

C5 C6 C7 …

Nivolumab alone: 240 mg intravenous (IV) every two weeks or 480 mg IV every four weeks. Continue until progression, unacceptable toxicity or 24 months, whichever comes first; the team confirms how this applies to your course.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

Testing across both treatment phases

Your team books tests and reviews their results. Imaging and tumour-marker timing is individualised; no universal interval is mandated here.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00900 v1a official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Nivolumab + ipilimumab · first-line colorectal: treatment and checks

First-line unresectable or metastatic MSI-H/dMMR colorectal cancer

Cycles 1–4: nivolumab 240 mg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg IV every two weeks or 480 mg IV every four weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Combination cycles 1–4	Transition to nivolumab alone	Maintenance	After treatment
Blood count, kidneys, liver, glucose and thyroid NCCP 00900	Baseline results	Before every cycle	Review results and symptoms before changing phase	Before every maintenance cycle	Follow persistent abnormalities
Symptoms and other hormones NCCP 00900	Discuss autoimmune disease, infections, medicines and earlier immune treatment	Assess immune symptoms; cortisol if indicated	A phase change does not remove immune risks	Report symptoms immediately; extra tests as indicated	Monitoring for at least five months after the last dose
Treatment and observation NCCP 00900	Confirm MSI-H/dMMR result and correct treatment line	Nivolumab first; then ipilimumab; vital signs and post-infusion observation	First maintenance dose three weeks after final combination	Confirm selected dose, route and two-/four-week interval	Retain the immunotherapy alert card

Care / check	Before starting	Combination cycles 1–4	Transition to nivolumab alone	Maintenance	After treatment
Scans and response NCCP 00900	Agree assessment plan	Scans and markers as directed	Review benefit and suitability for maintenance	Review benefit, tolerability and duration	Agree next care and follow-up

What each check is for

1 Cycles 1–4: two immunotherapies

Every 21 days

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This is a first-line treatment for unresectable or metastatic cancer.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

2 Cycle 5 onwards: nivolumab alone

Team-selected maintenance schedule

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This is a first-line treatment for unresectable or metastatic cancer.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Combination appointments

During the first four cycles, nivolumab is infused first over about 30 minutes, then ipilimumab over about 30 minutes. Allow extra time for checks and observation. Nivolumab alone: 240 mg IV every two weeks or 480 mg IV every four weeks.

STEP 2

Changing phase

STEP 3

Tests and reviews

Before starting and before every cycle: blood count, kidney and liver profile, glucose and thyroid function. Cortisol is checked if clinically indicated. Disease assessment follows the consultant plan. Continue monitoring for late immune effects for at least five months after the final dose.

STEP 4

Alert card and home care

There is no take-home chemotherapy pump. Keep the card, medicine list, symptom record and urgent contact numbers accessible.

STEP 5

Treatment duration

Continue until progression, unacceptable toxicity or 24 months, whichever comes first; the team confirms how this applies to your course.

Combination first, then nivolumab alone

Keep both phases in your written plan.

Four combination cycles

Nivolumab 240 mg and ipilimumab 1 mg/kg are given on Day 1 every 21 days for four cycles. Nivolumab is infused first, then ipilimumab; each infusion takes about 30 minutes, with additional preparation and observation.

Tests and treatment reviews

Checks continue in both phases.

Blood and thyroid tests

Scans and markers

The supervising consultant determines imaging, tumour-marker and other disease-monitoring tests. Ask when each test is due and who will review the result.

Treatment delays

If a dose is held, the team decides whether and when either medicine can restart. Do not assume one medicine is safe to receive when the other has been withheld.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Combination treatment	 **Day 1** Day 1 in cycles 1–4: nivolumab then ipilimumab
Immune effects	 **Days 1–21** Any day; also during maintenance or after stopping
Symptoms and tests	 **Days 1–21** Blood and thyroid tests before each treatment cycle

This 21-day calendar describes the first four combination cycles. Maintenance uses the separately confirmed two- or four-week schedule.

The next cycle starts 21 days after this one, if your team confirms it.

> Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell every healthcare team that you have received nivolumab and ipilimumab.

Between-treatment notes

Day 1: Combination visit: cycles 1–4

Blood results and symptoms are reviewed. Nivolumab is given first, then ipilimumab. The team monitors you during treatment and afterwards. Tell the nurse immediately if you become itchy, flushed, breathless, dizzy or suddenly unwell.

Day 2–21: Between combination visits

Immune inflammation can begin at any time; the calendar cannot predict a safe symptom-free week. After cycle 4, confirm the nivolumab-only dose, route, interval and first maintenance date.

Support around the medicines

1 Sickness prevention and premedication

Nivolumab has minimal and ipilimumab low emetogenic potential. Routine premedication is not usually required; the team individualises anti-sickness treatment and care after any previous reaction.

2 Steroids and other treatment for immune effects

Do not self-start or stop steroids. If prescribed, follow the exact taper and any infection-prevention plan. Do not restart cancer treatment until the team explicitly authorises it.

3 Pregnancy and feeding

Discuss fertility and effective contraception before treatment. Effective contraception continues for at least five months after the last nivolumab dose. Do not breastfeed during treatment; agree any later feeding plan with the team.

Between visits and practical support

Keep an early-reporting plan.

Bowels and hydration

Report new diarrhoea, blood in stools or abdominal pain immediately. Immune bowel inflammation can worsen and needs assessment; do not rely on self-treatment alone.

Energy, hormones and daily life

New marked fatigue, headache, dizziness, weakness, thirst or extra urine can need hormone/glucose tests. Ask for help with eating, activity, sleep, work and caring responsibilities.

Medicine list and steroids

Tell the team about all medicines, supplements and steroid use. Follow any steroid taper exactly and carry information about hormone replacement.

Urgent symptoms and care after treatment

Do not wait for the next appointment.

During infusions

Alert the nurse immediately if you feel itchy, flushed, breathless, dizzy, tight-chested or suddenly unwell.

Late effects

Keep the alert card and report symptoms after treatment stops. NCCP specifies continuous monitoring for at least five months after the last dose. Some hormone changes need longer follow-up.

Questions worth asking

undefined

undefined

undefined

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undefined

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undefined

undefined

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Colorectal treatment setting First-line unresectable or metastatic MSI-H/dMMR colorectal cancer. Other nivolumab/ipilimumab regimens have different doses, routes and timing.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycles 1–4: nivolumab + ipilimumab every 21 days

Active medicines: Nivolumab + Ipilimumab

Emetogenic risk: Low combination; nivolumab minimal, ipilimumab low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before a combination treatment day, if prescribed under the low-risk antiemetic plan NCIS V8 low-risk reference, distinct from infusion premedication. Review QT risk, electrolytes and interactions.
Metoclopramide	10 mg orally up to three times daily as prescribed	As-needed rescue on subsequent days Confirm contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Routine infusion premedication is not usually required. Antiemetics above are a reference requiring individual/local prescription; do not add prophylactic corticosteroids automatically.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.

- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Confirm nivolumab first, separate infusion sets/filters and source-specific observation. The weight-based 00551 regimen and flat-dose 00900 regimen are not interchangeable.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Cycle 5 onwards: prescribed nivolumab monotherapy schedule

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or infusion premedication; prescribe symptom relief individually.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.
- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Cycles 1–4: nivolumab 240 mg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg intravenous (IV) every two weeks or 480 mg IV every four weeks.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / nervous system

Nivolumab + ipilimumab

New severe headache, dizziness, confusion, weakness, thirst or extra urine needs urgent advice; hormone effects may need long-term treatment.

3 Heart

Nivolumab + ipilimumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

5 Kidneys

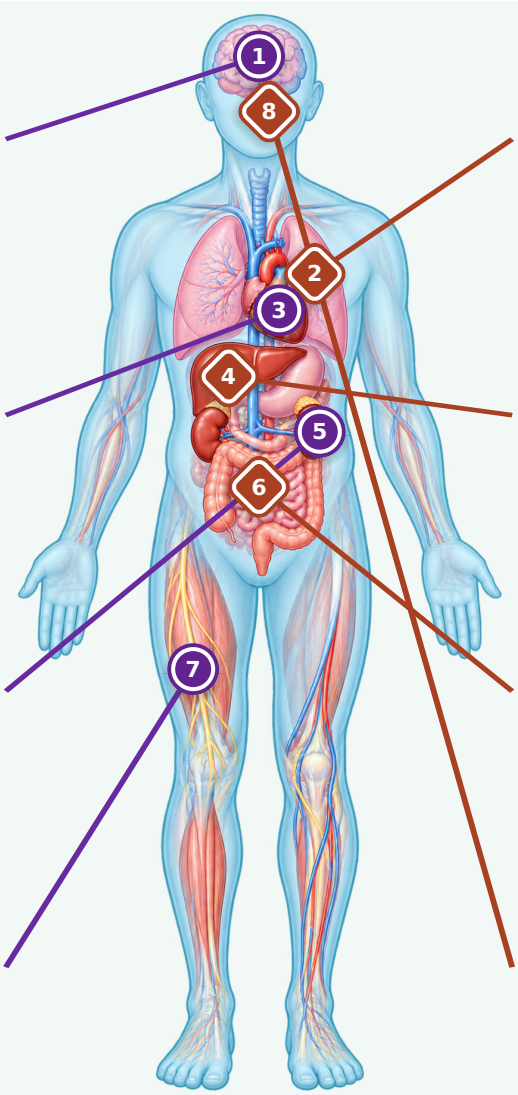
Nivolumab + ipilimumab

Report much less urine, new swelling or a change in kidney tests.

7 Skin / muscles

Nivolumab + ipilimumab

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.



2 Lungs

Nivolumab + ipilimumab

New cough or breathlessness can signal inflammation or another urgent cause.

4 Liver

Nivolumab + ipilimumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

6 Bowel

Nivolumab + ipilimumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

8 Eyes / vision

Nivolumab + ipilimumab

Report eye pain or new visual change promptly; do not wait for the next cycle.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for new breathlessness, persistent diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, spreading or blistering rash, severe headache, marked weakness, confusion, unusual thirst or passing much more urine. Severe breathing difficulty, chest pain, collapse or seizure needs emergency care. Effects can begin after treatment ends.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00900 v1a official regimen**
   https://healthservice.hse.ie/documents/6653/900_v1a_Nivolumab_240mg_ipilimumab_1mgkg_.pdf
2. **EMA Opdivo product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf
3. **EMA Yervoy product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf
4. **CheckMate 8HW randomised first-line colorectal trial**
   https://pubmed.ncbi.nlm.nih.gov/39602630/
5. **HSE treatment safety and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
7. **NCCP regimen; NCIS Medical Oncology antiemetics V8 Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf